

PDPM Overview for DORs

Patient Driven Payment Model · What it means for therapy · What you need to know · 2026

PDPM — the Patient Driven Payment Model — replaced the RUG-IV therapy minute-based payment system in October 2019. Under PDPM, Medicare Part A SNF payment is driven by the patient's clinical characteristics and functional status — not by therapy minutes. This fundamentally changes what therapy needs to focus on: **quality of outcomes, accurate functional assessment, and appropriate clinical classification.**

The Big Shift — RUG-IV vs PDPM

	RUG-IV (Before Oct 2019)	PDPM (Current)
Payment driven by	Therapy minutes per week	Patient clinical characteristics
Therapy incentive	Bill as many minutes as possible	Accurate assessment & outcomes
Min therapy required	Yes — Ultra High, Very High, etc.	No minimum therapy minutes
Section GG	Not used for payment	Directly drives PT & OT payment
Concurrent/Group limit	Counted toward minutes	No therapy minute threshold
PDPM components	Single therapy RUG tier	5 separate payment components

The 5 PDPM Payment Components

Each Part A day generates payment across 5 separate components. Each component has its own rate based on the patient's clinical profile. The total per diem rate = sum of all 5 components.

PT Component — Physical Therapy

What drives payment:	Therapy implication:
Clinical category (based on primary diagnosis) + Section GG functional score at admission. Lower Section GG scores (more dependent) = higher PT payment. Rate varies by clinical category: Major Joint Replacement, Hip Fracture, Stroke, Pulmonary, Non-Ortho Surgery, Orthopedic, and others.	Accurate Section GG admission scoring is critical. Under-scoring (marking patient more independent than they are) reduces PT payment. Evaluate within 3 days and document functional performance observed — not best performance.

OT Component — Occupational Therapy

What drives payment:	Therapy implication:
Same clinical category as PT + Section GG functional score at admission. OT payment mirrors PT payment structure — both are driven by diagnosis category and functional dependence at admission.	Same as PT — accurate Section GG scoring directly impacts OT revenue. OT eval should capture ADL functional status accurately within the 3-day assessment window. Do not score based on what the patient can do at their best — score typical performance.

SLP Component — Speech-Language Pathology

What drives payment:	Therapy implication:
Presence of SLP-relevant comorbidities (aphasia, dysphagia, TBI, etc.), cognitive function score (BIMS), and swallowing disorder on MDS. Does NOT use Section GG scores.	Ensure SLP diagnoses are accurately captured in the MDS. BIMS cognitive score and swallowing disorder codes must be coded correctly. Missed SLP diagnoses on MDS = lost SLP payment even if patient is being treated.

Nursing Component

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What drives payment:	Therapy implication:
Nursing classification based on primary diagnosis, comorbidities, and care needs identified by nursing on the MDS.	Not directly driven by therapy. However, accurate therapy documentation of functional status informs MDS coding, which affects the nursing component. Ensure your eval captures all relevant comorbidities.

Non-Therapy Ancillary (NTA)

What drives payment:	Therapy implication:
High-cost conditions and services — IV medications, complex wound care, dialysis, parenteral nutrition, respiratory therapy. Driven by qualifying conditions on MDS.	Not therapy-driven. NTA rates are highest in the first days of the stay and taper down. Relevant to understand why high NTA patients may have a higher facility per diem regardless of therapy intensity.

Section GG — The Most Important Thing for PT/OT Payment

What it is	A functional assessment of 16+ activities scored on a 01–06 scale (01=Dependent, 06=Independent). Completed at admission and discharge by therapy and nursing.
Admission score	Captured in the first 3 days of the stay. Lower scores (more dependent) = higher PT and OT component payment. Score must reflect OBSERVED performance — not estimated or best-case.
Discharge score	Captured in the last 3 days of the stay. Used for QRP quality reporting. Missing discharge scores = 2% Medicare payment penalty for the facility.
Who completes it	Therapy (PT, OT, ST) and nursing both contribute. Therapy scores mobility and self-care. MDS coordinator assembles the final MDS — communicate your scores in writing before the MDS locks.
Biggest DOR mistake	Scoring the patient at their best performance or their prior level. Section GG must reflect what you actually observed during the assessment window, not what they could do at home. Inflated scores reduce PT and OT payment.

What This Means for You — DOR Action Items

Evaluate within 72 hours	PDPM payment starts on day 1. Late evals mean the MDS may be completed before therapy input is captured. Eval on day 1 or 2 whenever possible.
Score Section GG accurately	This is the single biggest lever you have over PT and OT payment. Score what you observe — lower functional scores = higher payment. Never inflate scores to make a patient look better than they are.
Communicate GG scores to MDS in writing	Don't assume the MDS coordinator has your scores. Submit your Section GG findings in writing before the MDS assessment reference date.
Verify SLP diagnoses are coded on MDS	If you have an SLP treating for aphasia, dysphagia, or cognitive-comm — confirm those diagnoses are coded on the MDS. Missing codes = lost SLP payment.
Document clinical complexity	Clinical category is assigned by the MDS from the patient's primary diagnosis. Ensure the admitting diagnosis reflects the most resource-intensive condition.
Track per diem vs length of stay	PDPM per diem rates vary throughout the stay (variable rates). Understand that longer stays have lower per diem rates — discharge planning matters.

What Most DORs Don't Know About PDPM

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Variable Per Diem — PT/OT Rates Step Down After Day 20

PT and OT payment rates are NOT flat across the entire stay. Under PDPM's variable per diem adjustment, PT and OT component rates are reduced after day 20 of the Medicare Part A stay. This means the facility earns less PT and OT revenue per day for long-stay patients. **Implication:** Keeping a patient on Part A past day 20 without strong clinical justification reduces per-day revenue. Discharge planning should be proactive — transition to Part B, outpatient, or home when clinically appropriate around or before day 20 when possible.

The 5-Day MDS Sets Payment for the Entire Stay

The primary PDPM MDS assessment has a reference date in days 1–5 of the stay. This single assessment determines the patient's clinical category, Section GG score group, SLP comorbidities, and NTA score — and locks in the payment rate for the entire stay (unless an IPA is triggered). **Implication:** Evaluate on day 1 or 2. Submit your Section GG scores to the MDS coordinator before the assessment reference date closes. A late or inaccurate eval can lock the facility into a lower payment rate for the entire admission.

IPA — Interim Payment Assessment

An Interim Payment Assessment (IPA) is a new MDS that can be triggered when a patient has a significant change in condition — return from hospitalization, major functional decline, new acute diagnosis, or significant improvement. The IPA reclassifies the patient under PDPM and resets the payment rate going forward. **Implication:** When a patient returns from the hospital or has a major status change, notify the MDS coordinator immediately. A new eval and updated Section GG scores should be submitted so the IPA captures the patient's current clinical complexity. Missed IPA opportunities = lost revenue.

Clinical Categories — Not All Diagnoses Pay the Same

The patient's primary diagnosis determines their PDPM clinical category, which directly affects the PT and OT payment rate. Higher-paying categories include: Major Joint Replacement or Spinal Surgery, Acute Neurological, Non-Ortho Surgery, and Cardiovascular/Coagulation. Lower-paying categories include Medical Management and Non-Surgical Orthopedic. **Implication:** The admitting diagnosis must accurately reflect the most resource-intensive condition. If a patient was admitted post-stroke but the MDS only codes "weakness," they may be placed in a lower-paying category. Communicate diagnosis specificity to the MDS coordinator.

CMI — Case Mix Index

CMI is the average PDPM score across all Medicare Part A residents at your facility. A higher CMI means your patient population is more clinically complex on average, which translates to higher average daily payment. Leadership tracks CMI closely — it directly reflects the financial performance of the Part A program. **Implication:** Every accurate Section GG score, every correctly coded diagnosis, and every triggered IPA contributes to your building's CMI. Under-scoring and missed coding opportunities suppress CMI and cost the facility revenue without any clinical reason.

PDPM Rewards Quality — Functional Improvement Is a Public Measure

CMS designed PDPM to shift incentives from therapy volume to clinical quality. Functional improvement from admission Section GG to discharge Section GG is tracked as a publicly reported quality measure on the CMS Care Compare website. Facilities are rated on how much their patients improve functionally during their SNF stay. **Implication:** Accurate admission scoring + genuine functional improvement + accurate discharge scoring = better quality ratings. Inflating admission scores or deflating discharge scores distorts quality data and suppresses your facility's public rating. Score honestly, treat effectively, document improvement.

Accurate clinical classification and Section GG scoring do. Your job is to evaluate early, score accurately, communicate to the MDS team, and deliver high-quality skilled care that produces measurable functional improvement.